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## 1. Executive Summary

**The context:** Cholera remains a major and preventable public health threat in Mozambique, where recurrent outbreaks are driven by limited access to safe water, sanitation and hygiene (WASH), constrained health services, and frequent climate related shocks such as floods and cyclones. The disease is endemic, with annual outbreaks during the rainy season—particularly in Cabo Delgado, Nampula, Sofala and Tete provinces—and has escalated significantly in recent years. From September 2022 to July 2024, the country experienced its largest outbreak in two decades, with more than 48,700 cases reported across 93 districts, followed by continued transmission through 2025.

**The objective:** The objective of this framework is to enable rapid, coordinated anticipatory action to reduce the scale and severity of potential large, out-of-the-ordinary cholera outbreaks in Mozambique. By acting during the early stages of abnormal transmission patterns—before outbreaks escalate—partners aim to prevent large-scale spread, reduce mortality, and protect vulnerable communities.

**The development process:** The framework was developed through a participatory process led by the Resident and Humanitarian Coordinator. With support from OCHA, its development involved the Ministry of Health (MISAU), the National Institute of Health (INS), UNICEF, and WHO. Clusters and local partners such as the Mozambique Red Cross were also consulted.

**The trigger:** The trigger mechanism relies on weekly epidemiological surveillance data shared by national health authorities. Complex models that attempt to predict cholera outbreaks are not yet reliable enough to serve as triggers in this setting. There are various limitations such as predictive uncertainty that may reduce their suitability for operational decision-making. The present trigger is therefore based on case data of acute watery diarrhea (AWD) at the district and province levels as well as case confirmation to anticipate large-scale outbreaks, estimated using historical data. Thresholds were defined through retrospective analysis using normalized incidence rates and population-based denominators. Thresholds will be reviewed and recalibrated every two years based on updated epidemiological evidence. The trigger is reached if:

### District-level criteria

- At least 2 districts in the same province exceed thresholds for the same 3 consecutive weeks. A district threshold is reached if:
  - Weekly AWD cases are at least 4 times the value of the previous week OR
  - Weekly AWD cases exceed the province-specific 99th percentile (percentile set based on district-level data for each province)
- In both cases, weekly AWD cases must be at least 100 (for non-capitals) or at least 300 (for capitals).

### Province-level criteria

- The Provincial total is more than 2,500 AWD cases per week for 3 consecutive weeks.

### Disease confirmation

- A cholera outbreak must be confirmed or classified as probable.

**The action plan:** Once a trigger is reached, partners implement a coordinated set of anticipatory actions aimed at reducing transmission and preventing escalation. Priority interventions focus on four pillars:

- **Surveillance and laboratory support:** Ensure cases are detected early and can be referred for further treatment.
- **Case management, Infection Prevention and Control (IPC):** Reduce the case fatality rate by providing medical treatment and ensure appropriate IPC measures in health facilities.
- **Water, Sanitation and Hygiene (WASH):** Reduce cholera outbreaks and transmission risk by ensuring access to safe water, appropriate sanitation, and promoting positive hygiene behaviors in vulnerable communities.
- **Risk Communication and Community Engagement:** Strengthen community engagement and communication to promote prevention and care-seeking behaviors.

Implementation is coordinated through existing national and provincial cholera response mechanisms. An anticipatory action task force— comprised of UNICEF, WHO, and OCHA—meets upon activation to define the geographical scope and guide implementation. Activities can be expanded to neighboring provinces if justified. All activities are expected to be completed within three months of activation.

**The financing:** The Emergency Relief Coordinator (ERC) has pre-arranged up to US\$1.5 million through the Central Emergency Response Fund (CERF) to support this framework. CERF disburses funds once:

1. The RC/HC endorses the framework and CERF application package
2. The ERC endorses the same package
3. The pre-agreed trigger is reached

Each activation releases US\$750,000, allowing for two activations within the two-year validity period. Funds are disbursed on a no regrets basis to ensure rapid action. Partners will provide co-funding through their internal mechanisms.

**The learning:** The present framework has incorporated several lessons learned from cholera AA activations in the Democratic Republic of Congo. In case of an activation, an after-action review will be conducted to gather further lessons learned from the implementation of the framework. Partners also commit to engage in more comprehensive learning efforts focused on measuring impact.

## 4. Cholera in Mozambique and anticipatory action

### Exposure and impact

#### Exposure

Cholera remains one of the most devastating yet preventable global public health threats, disproportionately affecting vulnerable populations in fragile settings. Cholera, which, if untreated can be fatal, is primarily driven by a lack of access to proper water, sanitation and hygiene, as well as insufficient health services. These often occur in the context of conflict, displacement or climate hazards such as floods, drought or cyclones<sup>1</sup>. In Mozambique cholera is endemic, with outbreaks reported every year during the rainy season from October-April, especially in Cabo Delgado, Nampula, Sofala and Tete provinces<sup>2</sup>, often linked to cyclones or floods.

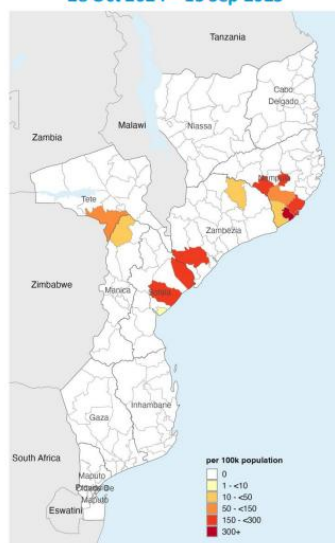
#### Humanitarian impact

Between September 2022 and July 2024, Mozambique saw a significant cholera outbreak, recording more than 48,751 cases in 93 districts - the most extensive outbreak in the last two decades. The outbreak, which occurred in the context of a global upsurge in cholera, remains active and is included in the WHO global classification of cholera as a Protracted Grade 3 emergency, reflecting its prolonged duration, wide geographic spread, and the high level of coordination required for effective response. Following a short period with no reported cases, from 28 October 2024 to 28 September 2025, a total of 4,568 cholera cases were reported across 13 districts of Mozambique, indicating sustained transmission and ongoing public health risk. This underscores the need for continued multisectoral engagement, enhanced surveillance, and sustained resource mobilization to address persistent vulnerabilities in water, sanitation, and hygiene (WASH) infrastructure and the need to strengthen case management capacity to prevent further escalation of the outbreak. The situation in Mozambique reflects a broader outbreak dynamic in the wider Southern and Eastern Africa region, where cholera is emerging as one of the most urgent public health emergencies.

### Anticipatory action for cholera

The potential to apply the anticipatory approach to infectious disease outbreaks is increasingly being recognized<sup>3</sup>. While ending cholera as a public health threat requires addressing its underlying causes, anticipatory action provides the opportunity to prevent a further escalation of outbreaks, reduce the loss of lives and livelihoods, and enable a more (cost-)effective and efficient response. This contributes to the Global Task Force on Cholera Control's (GTFFC) goals of reducing cholera deaths by 90%, eliminating cholera in as many as 20 countries, and preventing uncontrollable cholera outbreaks in fragile settings, as laid out in its global roadmap to 2030<sup>4</sup>.

Attack rate per 100 000 population by district,  
28 Oct 2024 – 15 Sep 2025



Source: WHO, Mozambique, Cholera excel sheets

<sup>1</sup> <https://www.who.int/news-room/fact-sheets/detail/cholera>

<sup>2</sup> <https://www.who.int/emergencies/disease-outbreak-news/item/2023-DON443>

<sup>3</sup> <https://www.anticipation-hub.org/learn/emerging-topics/health>

<sup>4</sup> <https://www.gtffcc.org/wp-content/uploads/2019/10/gtffcc-ending-cholera-a-global-roadmap-to-2030.pdf>

## 5. Coordination Arrangements

### Core group

The framework was developed under the leadership of the Resident and Humanitarian Coordinator by a core group comprised of UNICEF, WHO, OCHA, the Ministry of Health (MISAU), and the National Institute of Health (INS). In addition, relevant coordination fora, such as the Humanitarian Country Team (HCT) and the Inter-Cluster Coordination Group (ICCG), were consulted.

### Synergies with other initiatives

The Mozambique Red Cross is finalizing a simplified Early Action protocol specifically targeting cholera. Moreover, this framework complements existing AA plans for cyclones (incl. an OCHA-facilitated multi-sectoral framework), drought and floods in line with the government's multi-hazard approach. Many of these include WASH and Health elements which help mitigate the risk of cholera outbreaks.

### Alignment with Government Policy

The present AA framework for cholera has been developed in close collaboration with relevant government authorities and will contribute to National Standard Operating Procedures for Anticipatory Action against cholera in Mozambique.

## 6. The Trigger Mechanism

### Epidemiological surveillance

National health authorities in Mozambique are responsible for collecting and validating epidemiological data. The Ministry of Health, WHO and OCHA have signed a memorandum of understanding to allow the data to be shared with the Center for Humanitarian Data for the purposes of regular monitoring in order to identify when any geographic area approaches the established trigger threshold. Aggregated data at the district level will be shared on a weekly basis to ensure timely activation should trigger thresholds be met. No personal or sensitive information will be shared as part of this project.

### Triggers and thresholds

A trigger for anticipatory action should be based on a set of criteria to help answer the questions of when and where to act before an imminent disaster. It should forecast when a hazard risks becoming an out-of-the-ordinary (or severe) shock and the humanitarian impact is likely to cross a certain threshold for the exposed vulnerable community. The trigger mechanism was defined through a participatory process with the various partners, including MISAU, INS, UNICEF, WHO, and the Centre for Humanitarian Data.

#### Triggering based on epidemiological indicators

Despite the many existing mathematical models that attempt to predict the onset of a cholera outbreak, these models are not yet reliable enough to serve as triggers in this setting. There are various limitations such as predictive uncertainty that may reduce their suitability for operational decision-making. The trigger is therefore based on epidemiological data at the district and province levels, to anticipate large-scale outbreaks, estimated using historical data.

Trigger activation is assessed using routinely reported surveillance data on a weekly basis, focusing on larger increases in reported cases of acute watery diarrhea (AWD) relative to historical baselines at both district and provincial levels. Thresholds are defined to identify periods where transmission exceeds expected variation, indicating an elevated risk of a large-scale outbreak. To reduce false alarms, the trigger looks at patterns of sustained transmissions, consecutive weeks of elevated caseloads and simultaneous increases across districts within a province. This approach allows the trigger to distinguish between localized fluctuations and broader escalations that would result in an increased public health risk.

Thresholds are defined through retrospective analysis of historical AWD surveillance time series. For each province, a single epidemiological threshold is established based on the historical distribution of weekly incidence rates, calculated using non-zero observations only to avoid downward bias associated with periods of zero reporting. Reported case counts are normalized by population and expressed as cases per 100,000 population, under the assumption that absolute caseloads are strongly correlated with population size. Population denominators are derived from the World Pop 2025 data, aggregated to admin boundaries. These thresholds will be reviewed and recalibrated on a biennial basis as additional epidemiological data becomes available. This framework is aligned with national surveillance systems anchored in observed epidemiological data.

#### The trigger is reached if:

- **District level:** At least 2 districts in the same province exceed thresholds for the same 3 consecutive weeks. A district threshold is reached if:
  - Weekly AWD cases are at least 4 times the value of the previous week OR
  - Weekly AWD cases exceed the province-specific 99th percentile (percentile set based on district-level data for each province).
  - In both cases, weekly AWD cases must be at least 100 (for non-capitals) or at least 300 (for capitals)<sup>5</sup>
- OR**
- **Province level:** The Provincial total is more than 2,500 AWD cases per week for 3 consecutive weeks.
- AND**
- A cholera outbreak must be confirmed or classified as probable (see below)<sup>6</sup>

<sup>5</sup> A capital district refers to the district that contains the provincial capital city.

<sup>6</sup> Confirmation is sought for each activation. If this is not feasible, a probable cholera outbreak, as defined by the GTFCC, is considered sufficient. This applies to both the district and province level triggers.

## Provincial Level Thresholds

Thresholds are defined by province due to the different epidemiological patterns by province and are normalized by population to allow for comparison across provinces.

| Province        | 99 <sup>th</sup> percentile threshold per 100k population |
|-----------------|-----------------------------------------------------------|
| Cabo Delgado    | 301.2                                                     |
| Gaza            | 267.1                                                     |
| Inhambane       | 127.5                                                     |
| Manica          | 74.8                                                      |
| Maputo City     | 226.2                                                     |
| Maputo Province | 136.5                                                     |
| Nampula         | 87.7                                                      |
| Niassa          | 272.5                                                     |
| Sofala          | 150.1                                                     |
| Tete            | 158                                                       |
| Zambezia        | 135.9                                                     |

## Definitions of probable and confirmed cholera outbreaks (GTFCC)<sup>7</sup>

  
**PROBABLE CHOLERA OUTBREAK**

A probable cholera outbreak is detected in a surveillance unit if within 14 days:

| Number of RDT+  | out of # suspected cases tested |
|----------------------------------------------------------------------------------------------------|---------------------------------|
| ≥ 3 RDT+                                                                                           | 3-7                             |
| ≥ 4 RDT+                                                                                           | 8-10                            |
| ≥ 5 RDT+                                                                                           | 11-14                           |
| ≥ 6 RDT+                                                                                           | 15-17                           |
| ≥ 7 RDT+                                                                                           | 18-21                           |

A probable cholera outbreak is detected when:

- the number of suspected cholera cases with a positive rapid diagnostic test (RDT+) achieves or surpasses a defined threshold (see graphic on the left), while taking into account the number of suspected cases tested.

  
**CONFIRMED CHOLERA OUTBREAK**

A cholera outbreak is confirmed when a surveillance unit has:

≥ 1    
at least 1 locally acquired confirmed case

A confirmed cholera outbreak is detected when:

- a surveillance unit has at least one locally acquired, confirmed cholera case.

## Parameters

This framework covers the **entire country** of Mozambique. One of its main objectives is to provide the necessary **flexibility** so partners can effectively respond to and contain an outbreak. If a district-level trigger is reached, partners may implement in the districts where the threshold was reached as well as in other parts of that province depending on outbreak dynamics.

<sup>7</sup> <https://www.gtfcc.org/wp-content/uploads/2025/04/public-health-surveillance-for-cholera-guidance-document-2024.pdf>

In the case of a provincial-level trigger, partners will agree on the areas of implementation through an AA task force. Partners will also be given some flexibility to use the available funding to respond in neighboring provinces (see “Coordination of implementation” in section 5 for details).

### Roles and responsibilities in monitoring the trigger and activation protocol

The Centre for Humanitarian Data will monitor the epidemiological data, which WHO will provide on a weekly basis, to determine whether a trigger threshold is reached. If the trigger is met, partners will be informed and CERF will release the funding, allowing partners to immediately launch their response. The details of the activation protocol can be found in the table below. Pre-defining this activation protocol is critical to avoid delays, miscommunication, uncertainty or errors once the trigger threshold is reached.

| When                                                  | Who                   | What                                                                                                                | How              |
|-------------------------------------------------------|-----------------------|---------------------------------------------------------------------------------------------------------------------|------------------|
| Weekly monitoring                                     | WHO                   | Shares surveillance data with CHD                                                                                   | Digital format   |
| When surveillance data indicates threshold is reached | CHD                   | Requests information from WHO that outbreak is confirmed or probable as per GTFCC criteria                          | Email            |
| When trigger is met                                   | CHD                   | Notifies RC/HC, OCHA (Country Office, Regional Office, HQ), CERF, UNICEF, WHO, MISAU, HCT, ICCG that trigger is met | Email            |
| After notification by CHD that trigger is reached     | CERF Secretariat      | Sends project approval letters to UN agencies                                                                       | Email            |
| After approval letters are sent                       | OCHA Mozambique       | Convenes coordination meeting within 24 hours to agree on geographical scope                                        | Meeting          |
| After coordination meeting                            | WHO, UNICEF, partners | Begin immediate project implementation                                                                              | As per framework |

## 7. The Delivery: Anticipatory Action Plan

### Selection criteria

Cholera response activities are well defined through global (e.g. by the GTFCC)<sup>8</sup> and national standard operating procedures. These include coordinated multi-sectoral interventions in the WASH and Health sectors as well as a strong emphasis on risk communication and community engagement. Interventions will aim to reduce disease transmission and provide treatment to patients in a timely manner, thereby preventing large-scale cholera outbreaks and reducing deaths. The following criteria were applied to select actions:

1. **Anticipatory character:** Is the action effective in preventing or reducing the humanitarian impact of the shock?
2. **Ability to deliver within the window of opportunity:** Is it possible to carry out the action effectively with the available forecast lead time, i.e., in the window of opportunity between the trigger and the anticipated peak of the outbreak?
3. **Operational capacity:** Do the agency and its implementing partners (IPs) have the institutional capacity (thematic, logistic, administrative, financial, human resources) to implement the action effectively given the lead time and scale?
4. **No regrets approach:** In the case of a false alarm, will the proposed actions benefit rather than negatively affect the targeted population?

### Priority interventions

The pre-agreed actions will focus on the following core pillars:

- **Surveillance and laboratory support:** Ensure cases are detected early and can be referred for further treatment.
- **Case management, Infection Prevention and Control (IPC):** Reduce the case fatality rate by providing medical treatment and ensure appropriate IPC measures in health facilities.
- **Water, Sanitation and Hygiene (WASH):** Reduce cholera outbreaks and transmission risk by ensuring access to safe water, appropriate sanitation, and promoting positive hygiene behaviors in vulnerable communities.
- **Risk Communication and Community Engagement:** Strengthen community engagement and communication to promote prevention and care-seeking behaviors.

The table in the Annex provides a detailed overview of each agency's activities. Further details can be found in CERF project documents.

### Coordination of implementation

Once a trigger is reached, partners will coordinate their implementation through established coordination mechanisms such as the national and provincial cholera task force and incident management system, ensuring a swift multi-sectoral response in the same geographical areas. Regular meetings of an anticipatory action task force will be convened to facilitate coordination, agree on the geographical scope at the district level, and monitor progress. The task force will be comprised of UNICEF, WHO, and OCHA. A framework activation enables partners to implement anywhere in the districts/province in which the trigger was met. The exact locations will be agreed during the task force meeting. Any changes in the geographical scope, including situations where agencies target certain regions individually, will be discussed and agreed upon by the task force. The expansion of activities to neighboring provinces is possible provided a clear rationale is provided and agreed by the task force.

### Timing

The objective of this framework is to mitigate the impact of predicted large-scale cholera outbreaks by enabling partners to respond immediately and without delay once a trigger is met. All agencies participating in this framework confirm their readiness to implement activities as soon as a trigger is reached. Partners are expected to implement all activities within three months after framework activation.

<sup>8</sup> <https://www.choleraoutbreak.org/>

## Ensuring readiness for a timely implementation

Agency-specific preparedness plans enabling a swift response are outlined below.

### UNICEF

Risk Communication and Community Engagement:

1. Routine data collection related to misinformation/rumors on cholera done through social media and community listening.
2. Regular promotion of hygiene practices related to cholera and diarrhea prevention, care seeking and treatment integrated into on-going community engagement, community radio and media platforms for health promotion.
3. Training and capacity building of community activists to be engaged in cholera response after declaration of outbreak.

WASH:

1. Prepositioning of critical emergency supplies for outbreak response.
2. Training of implementing partners, cluster members on WASH in Emergencies including outbreaks.
3. Establishment of contingency partnerships that can be activated for outbreak response.

### WHO

1. Regular monitoring of acute water diarrhea (AWD) surveillance data.
2. Regular development of WHO preparedness and response plans.
3. Implementation of cholera readiness assessments with the Ministry of Health.
4. Conducting risk assessments/public health situation analyses.
5. Grading with activation of Incident Management System (IMS).
6. Support the districts in updating their contingency plans.
7. Prepositioning of cholera kits and supplies in the provinces.

## 8. Pre-arranged Financing

### CERF

For this anticipatory action framework for cholera in Mozambique, the ERC has agreed to allocate *up to \$1.5 million* if triggers are reached. Further details can be found in the CERF application documents.

CERF will disburse funds once three conditions are met:

- Endorsement by the Resident/Humanitarian Coordinator of the country-specific Anticipatory Action framework and the CERF application package; and
- Endorsement by the Emergency Relief Coordinator of the country-specific Anticipatory Action framework and the CERF application package comprising an application chapeau, project proposals and budgets; and
- Activation of the framework by reaching the pre-agreed trigger

CERF will disburse funds on a no-regrets basis as soon as the trigger is reached<sup>9</sup>.

**Each activation prompts the release of \$750,000 from CERF**, i.e. a total of two activations is possible within the two-year validity period of this framework. Per activation, UNICEF will receive \$500,000 and WHO \$250,000.

Note: Having an anticipatory action framework supported by CERF does not preclude the RC/HC/ HCT from seeking additional CERF Rapid Response funding for the same or other shocks that may be experienced. At the same time, it does not guarantee it either.

### Co-financing

This framework for anticipatory action is not limited to CERF-funded activities. Both UN agencies will leverage internal mechanisms to scale up framework activations and complement CERF funding.

### Funding by agency

| Agency       | Sector | CERF funding (in US\$) | Preparedness co-funding (in US\$) | Implementation co-funding (in US\$) | Number of people targeted | Total (in US\$)  |
|--------------|--------|------------------------|-----------------------------------|-------------------------------------|---------------------------|------------------|
| UNICEF       | WASH   | 1,000,000              | 100,000                           | 150,000                             |                           | 1,250,000        |
| WHO          | Health | 500,000                | 115,000                           | 128,000                             |                           | 743,000          |
| <b>TOTAL</b> |        | <b>1,500,000</b>       | <b>215,000</b>                    | <b>278,000</b>                      |                           | <b>1,993,000</b> |

<sup>9</sup> While CERF approval letters will be sent out immediately following an activation, the administrative transfer of funds to agencies may be delayed by a few days. To avoid a delay in project implementation, agencies confirm their ability to frontload the resources required to initiate their response until the funding has been received.

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## 9. Learning

### Monitoring & Evaluation

Each agency will use its existing monitoring & evaluation systems to collect and track data on implementation progress and outputs achieved. All findings will be shared. In addition, discussions are underway on a collective learning agenda to better understand the benefits of the approach, with a special focus on the impact, timeliness and (cost-)effectiveness compared to a regular response.

### Lessons from other frameworks

This is the second coordinated anticipatory action framework for cholera, which allows for the incorporation of lessons from the first experience in the Democratic Republic of the Congo. Examples include:

- The integration of a provincial-level trigger
- Greater geographical flexibility
- Strengthening of awareness among implementing partners
- Clear coordination mechanisms

## 10. Annex

### Overview of cholera activities implemented by partners

| Sector/<br>Response pillar                                                   | Objective                                                    | Implementing<br>organization | Activities                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
|------------------------------------------------------------------------------|--------------------------------------------------------------|------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Health -<br>Surveillance and<br>laboratory support                           | Early<br>detection, case<br>referral                         | WHO                          | Distribution of rapid diagnostic test (RDT) kits and culture sample collection materials                                                                                                                                                                                                                                                                                                                                                                                                                                                                                      |
|                                                                              |                                                              |                              | Logistical support for the referral of samples to provincial laboratories                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|                                                                              |                                                              |                              | Deployment of Rapid Response Teams to facilitate case investigations, sample collection, training and other actions                                                                                                                                                                                                                                                                                                                                                                                                                                                           |
|                                                                              |                                                              |                              | One-day refresher training for community actors in community surveillance                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|                                                                              |                                                              |                              | Active case finding and follow-up                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
| Health - Case<br>Management,<br>Infection<br>Prevention and<br>Control (IPC) | Reducing case<br>fatality rate                               |                              | Distribution of cholera kits and medical supplies                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|                                                                              |                                                              |                              | Setup of Cholera Treatment Centers/Units (CTUs/CTCs) and Oral Rehydration Points (ORPs)                                                                                                                                                                                                                                                                                                                                                                                                                                                                                       |
|                                                                              |                                                              |                              | Refresher training for health staff on case management and Infection Prevention and Control (IPC)                                                                                                                                                                                                                                                                                                                                                                                                                                                                             |
|                                                                              |                                                              |                              | Recruitment of additional staff for case management and IPC in CTCs/CTUs/ORPs                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                 |
|                                                                              |                                                              |                              | Refresher training of community leaders/ actors in safe burials                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |
| Water, Sanitation<br>and Hygiene<br>(WASH)                                   | Reducing<br>cholera<br>outbreaks and<br>transmission<br>risk | UNICEF                       | Train WASH actors on cholera preparedness, hygiene promotion, and rapid response protocols                                                                                                                                                                                                                                                                                                                                                                                                                                                                                    |
|                                                                              |                                                              |                              | Local Procurement and distribution of Cholera Family Kits                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                     |
|                                                                              |                                                              |                              | <u>Safe water access</u> <ul style="list-style-type: none"><li>• Monitor and treat community water sources to ensure safe drinking water</li><li>• Maintenance of water supply systems and hand pumps</li><li>• Distribute water purification supplies and train households on proper use</li></ul>                                                                                                                                                                                                                                                                           |
|                                                                              |                                                              |                              | <u>Sanitation and Hygiene Promotion</u> <ul style="list-style-type: none"><li>• Promote construction or repair of household, communal and institutional latrines</li><li>• Promote safe disposal of human waste through community messaging and coordination with local authorities</li><li>• Mobilize volunteers/community activists to conduct community awareness campaigns on cholera prevention, handwashing, and health-seeking behaviors</li><li>• Improve infection prevention and control (IPC) measures in health facilities with allocation of activists</li></ul> |
|                                                                              |                                                              |                              | <u>Water Quality Surveillance</u> <ul style="list-style-type: none"><li>• Map and assess water sources for contamination risks and response prioritization for the specific outbreak</li><li>• Conduct water quality testing in priority locations</li></ul>                                                                                                                                                                                                                                                                                                                  |
|                                                                              |                                                              |                              |                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                                               |

|                                             |                                                                                          |  |                                                                                                                                                                                                                                                                                                                                           |
|---------------------------------------------|------------------------------------------------------------------------------------------|--|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| Risk Communication and Community Engagement | Strengthening community engagement and communication to promote prevention, care-seeking |  | <ul style="list-style-type: none"> <li>• Train activists and volunteers on Case Area Targeted Interventions (CATI)</li> <li>• Strengthening community-based surveillance</li> <li>• Disseminate key messages on prevention and health promotion through community radios, social mobilization and influential community actors</li> </ul> |
|                                             |                                                                                          |  | <ul style="list-style-type: none"> <li>• Ensure community engagement through community leaders in the prevention and control of cholera and other diarrheal diseases</li> </ul>                                                                                                                                                           |

## Objectives of anticipatory action

There is broad agreement on the need for the international humanitarian sector to move from a largely responsive approach to an anticipatory approach. An anticipatory approach leads to a more effective, efficient and dignified response. It also protects hard-won development gains.

Today, we can predict with increasing confidence the occurrence and humanitarian impact of certain climate- and weather-related shocks. By combining different analytical approaches, out-of-the-ordinary weather events can not only be predicted, but their projected impact can proactively be mitigated based on pre-identified anticipatory actions. Building on growing evidence that acting prior to the onset of a predictable hazard is significantly more (cost-) effective than traditional humanitarian response, OCHA has facilitated the setup of multiple Anticipatory Action frameworks.

Each framework comprises 3 core elements, all of which are underpinned by a clear learning, monitoring and evaluation plan:

- A robust forecasting embedded in a clear decision-making process (the **model**).
- Pre-agreed action plans that can fundamentally alter the trajectory of the crisis (the **delivery**).
- Pre-arranged finance (the **money**).

Anticipatory action is still an innovative space. Thus, in addition to the 3 core elements, OCHA also invests in documenting evidence and learning from each framework.



## Core principles

Each anticipatory action framework is tailored to the local context. OCHA has identified nine core principles for the framework in Mozambique:

- **Prioritizing a multi-sectoral approach:** Partners should prioritize multi-sectoral, co-targeted interventions when possible to maximize synergies and to achieve great impact.
- **Ensuring the centrality of protection:** In line with the IASC centrality of protection policy, and the principle of “leaving no one behind” (2030 Agenda), all partners implementing anticipatory interventions have the responsibility to ensure that their response will not aggravate the exposure of communities to risks of violence, insecurity, extortion and exploitation. Anticipatory actions must also ensure that they benefit all communities (local communities, displaced communities, returnees, and refugees) based on their needs, with due attention given to obstacles linked to gender, age, disability or social affiliation.
- **Promoting accountability to affected populations:** Partners engaged in the implementation of the anticipatory action framework are responsible for the integration of accountability to affected populations approaches in their activities. This includes making all efforts to solicit, hear, and act upon the voices and priorities of affected people (including the most marginalized and at-risk women, men, girls and boys) in a coordinated manner, before, during, and after anticipatory action. It also means ensuring that community feedback leads to corrective action in future anticipatory action.
- **Supporting localization:** All partners involved in the implementation of this framework are expected to honor the HCT’s commitment towards localization. Equitable partnerships with local actors building upon their long-term relationships and trust with communities during anticipatory action are crucial.
- **Engaging in partnerships:** Collaboration across humanitarian actors is key to ensuring that all sectors are engaged, and that assistance is mobilized according to the AA framework.
- **Generating development co-benefits:** Framework participants are encouraged to consider how financing for anticipatory action can complement financing for development by reducing suffering and addressing the root causes of problems. For instance, if the pre-agreed anticipatory action plan includes the repair of boreholes, implementing agencies should ensure that the quality of the repairs will last beyond the immediate humanitarian crisis.
- **Capturing learning:** Monitoring, evaluation and learning activities should be embedded at every phase of the development and implementation of the AA framework.
- **Striving for integration:** The framework seeks to integrate the anticipatory action activities into the existing humanitarian architecture and to foster linkages with long term programming, including resilience and development initiatives.

## Average Annual Spending

|                                                       | Provincial | District | Combined |
|-------------------------------------------------------|------------|----------|----------|
| Return Period (Full Envelope)                         | 4.7        | 7.0      | 2.8      |
| Average Annual Spending (given \$750k per activation) | 321,429    | 214,286  | 535,714  |